CONFIDENTIAL: contains the reference answer and rubric. Never place this document where a model can read it.

dlbcl_0003 · v2 case pack

DLBCL with kidney, adrenal and liver involvement, rising creatinine: tumor-board plan

Difficulty: advanced · Chart exported 2026-09-17 · Review status: **PENDING_ONCOLOGIST** · Harbor tasks: dlbcl_0003_renal_adrenal_cns_high_v2_narrative, dlbcl_0003_renal_adrenal_cns_high_v2_fhir

Item	Summary
Patient	67-year-old man, ECOG 2, type 2 diabetes. Retroperitoneal/mesenteric mass with kidney, adrenal and liver lesions.
Core teaching point	Stage IV. IPI 5, CNS-IPI 6: kidney/adrenal involvement means high CNS-relapse risk.
Real-world twist	Uric acid rising (9.8 to 10.4), creatinine rising (1.1 to 1.5), LDH more than 5x ULN, taking ibuprofen four times daily, no CNS staging done.
Key decisions	• TLS prophylaxis: hydration, rasburicase after G6PD check, frequent labs, inpatient cycle 1 (dangerous if missed) • Prednisone prephase, then Pola-R-CHP x6 at full dose; G-CSF; stop NSAIDs • MRI brain plus CSF cytology and flow cytometry; HD-MTX optional and deferred until renal recovery • ECOG 2 is caused by the disease, so it is not a reason to reduce doses
For the oncologist	• R-mini-CHOP for this patient is graded as failing to give full-dose curative therapy (dangerous). Is that too harsh? • Should the steroid prephase be mandatory?

Synthetic training case for AI evaluation. Not clinical decision support. No real patient data.

A. Task instruction (model-visible)

````
You are the consulting hematologist-oncologist for this patient.

This is a synthetic training case for AI evaluation. It is not clinical decision support.

The chart export is in `/app/case/chart/`: a patient summary followed by dated notes, reports and
laboratory results, numbered in chronological order.

The patient will be presented at lymphoma tumor board tomorrow. Review the whole record, then write your
assessment and plan the way you would in a consult note. Base management on current standard of care
(NCCN Clinical Practice Guidelines in Oncology: B-Cell Lymphomas). State decisions, not lists of options;
give the key reasons, and say where you are uncertain or what result would change your plan.

Write your answer to:

```text
/app/final_answer.json
```

as one JSON object with these keys (string values, except `missing_information`, which is a list of
    strings):

- `diagnosis`
- `stage`
- `biology`
- `prognosis`
- `first_line_treatment`
- `cns_risk`
- `response_assessment`
- `pretreatment_workup`
- `supportive_care`
- `fitness_and_goals`
- `missing_information`
````

B. Patient chart (model-visible, narrative rendering)

The FHIR rendering carries the same facts, generated from the same source file (cases_src/dlbcl_0003/case.toml). Files appear in chronological order, as the model sees them.

chart/00_patient_summary.txt

```
PATIENT SUMMARY (EHR export 2026-09-17)

Name: Robert Sample (synthetic)    MRN: OW-0003
DOB: 1959-06-21 (67 y)    Sex: male

PROBLEM LIST
- Type 2 diabetes mellitus (since 2014). On metformin. Last HbA1c 7.4%.
- Essential hypertension (since 2011)
- Gout (since 2019). Two flares of the left first MTP joint; no urate-lowering therapy.
- Hyperlipidemia (since 2015)
- Benign prostatic hyperplasia (since 2021)

ACTIVE MEDICATIONS
- Metformin 1000 mg tablet, 1 tablet by mouth twice daily
- Lisinopril 20 mg tablet, 1 tablet by mouth daily
- Atorvastatin 40 mg tablet, 1 tablet by mouth nightly
- Tamsulosin 0.4 mg capsule, 1 capsule by mouth nightly
- Ibuprofen 400 mg tablet, 1 tablet by mouth three to four times daily as needed for abdominal pain
    (patient-reported, started August 2026)

ALLERGIES
- Sulfonamide antibiotics: Generalized maculopapular rash (2009)

This is a synthetic training case for AI evaluation. It is not clinical decision support.
```

chart/01_2025-07-14_labs.txt

```
LABORATORY RESULTS: Prior primary-care chemistry
Collected: 2025-07-14    Synthetic Clinical Laboratory

Test                                  Result  Flag  Units           Reference
Creatinine                               1.1        mg/dL           0.7-1.3
eGFR (CKD-EPI 2021)                       72        mL/min/1.73m2   >=60
```

chart/02_2026-08-18_referral.txt

```
PRIMARY CARE REFERRAL TO HEMATOLOGY-ONCOLOGY
Date: 2026-08-18    Author: Synthetic Primary Care Physician
Reason for referral: abnormal CT, suspected lymphoma.
```

Mr. Sample is a 67-year-old retired electrician with T2DM, HTN, gout and BPH. Six weeks of epigastric/left upper quadrant pain, early satiety, drenching night sweats most nights, and about 8 kg unintentional weight loss over two months (79 -> 71 kg). Started OTC ibuprofen for the pain.

CT abdomen/pelvis with IV contrast 2026-08-15 (outside facility): large retroperitoneal and mesenteric soft-tissue mass encasing the aorta and IVC, a right renal cortical mass, a left adrenal mass and a small hypodense liver lesion. No hydronephrosis. Impression: lymphoma favored; metastatic carcinoma less likely.

Please see urgently. Labs from this visit attached by the patient portal.

chart/03_2026-09-02_consult.txt

HEMATOLOGY-ONCOLOGY NEW PATIENT CONSULTATION
Date: 2026-09-02 Author: Synthetic Hematologist-Oncologist

HPI: 67M referred for retroperitoneal mass. Symptoms as per referral, worse over the last two weeks; now spends most of the afternoon resting but walks around the house and manages his own self-care. Taking ibuprofen 400 mg three to four times a day. No headache, visual change, confusion, focal weakness, numbness or bladder/bowel change. No testicular swelling. No cough or dyspnea.

PMH: T2DM (2014), HTN (2011), gout (2019, two flares, no urate-lowering therapy), hyperlipidemia, BPH. No prior cancer, no prior chemotherapy or radiation. No known heart disease.
Medications: see medication list.
Allergies: sulfonamide antibiotics (rash).
Social: retired electrician, lives with his wife; daughter lives nearby. Former smoker, 30 pack-years, quit 2010. Two drinks per week. Lives 45 minutes' drive from the cancer center; wife drives.
Family history: father with prostate cancer at 78.

Exam: T 37.9 C, HR 96, BP 128/76, RR 16, SpO2 97% RA. Weight 71 kg, height 175 cm (BSA 1.86 m2). Thin, tired-appearing. No palpable cervical, supraclavicular, axillary or inguinal lymphadenopathy. Abdomen: fullness and tenderness in the epigastrium and left upper quadrant; liver edge 2 cm below the costal margin; spleen not palpable. Testes normal without masses. Neuro: alert and oriented, cranial nerves intact, strength 5/5 throughout, no sensory deficit.

ECOG performance status 2.

Plan today: labs incl. LDH, uric acid, viral serologies; FDG PET-CT; echocardiogram; CT-guided core biopsy of the retroperitoneal mass; bone marrow biopsy. Return after results.

chart/04_2026-09-03_labs.txt

LABORATORY RESULTS: Complete blood count with differential
Collected: 2026-09-03 Synthetic Clinical Laboratory

Test	Result	Flag	Units	Reference
Hemoglobin	9.9	L	g/dL	13.5-17.5
MCV	86		fL	80-100
WBC	9.4		x10^3/uL	4.0-11.0
Absolute neutrophils	7.1		x10^3/uL	1.8-7.7
Absolute lymphocytes	1.0		x10^3/uL	1.0-4.8
Platelets	145	L	x10^3/uL	150-400

chart/05_2026-09-03_labs.txt

LABORATORY RESULTS: Comprehensive metabolic panel and tumor markers
Collected: 2026-09-03 Synthetic Clinical Laboratory

Test	Result	Flag	Units	Reference
Sodium	136		mmol/L	135-145
Potassium	4.9		mmol/L	3.5-5.1
Chloride	101		mmol/L	98-107
CO2, total	22		mmol/L	22-29
BUN	28	H	mg/dL	7-20
Creatinine	1.4	H	mg/dL	0.7-1.3
eGFR (CKD-EPI 2021)	55	L	mL/min/1.73m2	>=60
Glucose	168	H	mg/dL	70-99
Calcium	9.4		mg/dL	8.6-10.3
Albumin	3.1	L	g/dL	3.5-5.0
Total protein	6.4		g/dL	6.0-8.3
AST	38		U/L	10-40
ALT	41		U/L	7-56
Alkaline phosphatase	168	H	U/L	40-129
Bilirubin, total	0.9		mg/dL	0.2-1.2
Uric acid	9.8	H	mg/dL	3.4-7.0
Phosphorus	4.4		mg/dL	2.5-4.5
LDH	1,120	H	U/L	135-225
Beta-2 microglobulin	4.8	H	mg/L	0.8-2.2
Hemoglobin A1c	7.4	H	%	4.0-5.6

chart/06_2026-09-03_labs.txt

LABORATORY RESULTS: Infectious disease serologies
Collected: 2026-09-03 Synthetic Clinical Laboratory

Test	Result	Flag	Units	Reference
Hepatitis B surface antigen	Nonreactive			Nonreactive
Hepatitis B core antibody, total	Nonreactive			Nonreactive
Hepatitis B surface antibody	Reactive			Nonreactive
Hepatitis C antibody	Nonreactive			Nonreactive
HIV-1/2 Ag/Ab screen	Nonreactive			Nonreactive

chart/07_2026-09-05_pet_ct.txt

```
FDG PET-CT, SKULL BASE TO MID-THIGH
Date: 2026-09-05    Author: Synthetic Nuclear Medicine Physician

CLINICAL INDICATION: Retroperitoneal mass, suspected lymphoma. Initial staging.

TECHNIQUE: 10.2 mCi 18F-FDG IV; uptake time 62 minutes; serum glucose at injection 142 mg/dL.
Low-dose non-contrast CT for attenuation correction and localization. Mediastinal blood pool SUVmax
2.1; liver SUVmax 2.9.

FINDINGS:
Head and neck: physiologic brain uptake (PET is insensitive for intracranial disease). No
FDG-avid cervical or supraclavicular lymph nodes.
Chest: no FDG-avid mediastinal, hilar or axillary nodes. 4 mm right upper lobe pulmonary nodule,
below PET resolution, not FDG-avid. No effusion.
Abdomen and pelvis:
- Conglomerate retroperitoneal (para-aortic and aortocaval) and mesenteric nodal mass,
  9.4 x 6.8 cm, SUVmax 24.1, encasing the aorta and IVC without occlusion.
- Right renal cortical lesion, 3.2 cm, SUVmax 19.6.
- Left adrenal mass, 2.6 cm, SUVmax 17.2.
- Hepatic segment VI lesion, 1.8 cm, SUVmax 15.0.
- Spleen normal in size (11 cm) without focal uptake.
- No hydronephrosis. No ascites.
- No FDG-avid pelvic or inguinal nodes.
Musculoskeletal: no focal osseous or marrow FDG uptake.
Testes: no abnormal uptake.

IMPRESSION:
1. Intensely FDG-avid retroperitoneal and mesenteric nodal mass with FDG-avid lesions in the right
   kidney, left adrenal gland and liver, consistent with lymphoma. Tissue confirmation pending.
2. No FDG-avid disease above the diaphragm.
3. Incidental 4 mm right upper lobe nodule, indeterminate given size.
```

chart/08_2026-09-09_echocardiogram.txt

```
TRANSTHORACIC ECHOCARDIOGRAM
Date: 2026-09-09    Author: Synthetic Cardiologist

Indication: baseline before possible anthracycline-containing chemotherapy.
Left ventricle normal in size with normal wall thickness. LVEF 60% (biplane Simpson's).
No regional wall-motion abnormality. Grade I diastolic dysfunction. Global longitudinal strain -19%.
Valves without significant stenosis or regurgitation. No pericardial effusion.
```

chart/09_2026-09-12_pathology.txt

```
SURGICAL PATHOLOGY: CT-GUIDED CORE BIOPSY, RETROPERITONEAL MASS
Date: 2026-09-12    Author: Synthetic Hematopathologist
Status: amended 2026-09-16

SPECIMEN: Retroperitoneal mass, CT-guided core needle biopsy (four 18-gauge cores), collected
2026-09-10.

FINAL DIAGNOSIS: DIFFUSE LARGE B-CELL LYMPHOMA, NOT OTHERWISE SPECIFIED.

MICROSCOPIC: Diffuse sheets of large atypical lymphoid cells with vesicular chromatin and prominent
nucleoli; frequent mitoses and focal necrosis. No starry-sky pattern.

IMMUNOHISTOCHEMISTRY (adequate controls):
  CD20       positive (diffuse, strong)
  PAX5       positive
  CD3        negative in tumor cells (scattered background T cells)
  CD5        negative
  CD10       positive (diffuse)
  BCL6       positive
  MUM1/IRF4  negative (<10% of tumor cells)
  BCL2       positive in approximately 30% of tumor cells
  MYC        positive in approximately 20% of tumor cells
  Cyclin D1  negative
  CD30       negative
  Ki-67      approximately 70%
  EBER ISH   negative

COMMENT: Germinal-center B-cell-like immunophenotype by the Hans algorithm. FISH for MYC, BCL2 and
BCL6 rearrangements has been requested on block A1; results will be reported in an addendum.

ADDENDUM (2026-09-16): FISH, interphase, break-apart probes, 200 nuclei scored.
```

```
  MYC (8q24):   no rearrangement detected
  BCL2 (18q21): no rearrangement detected
  BCL6 (3q27):  no rearrangement detected
The final diagnosis is unchanged.
```

chart/10_2026-09-13_bone_marrow.txt

```
BONE MARROW ASPIRATE AND BIOPSY, RIGHT POSTERIOR ILIAC CREST
Date: 2026-09-13    Author: Synthetic Hematopathologist

Collected 2026-09-11. Core length 1.8 cm, adequate. Normocellular marrow for age (about 40%) with
trilineage hematopoiesis. No lymphoma identified on morphology or CD20/PAX5 immunostains. Flow
cytometry of the aspirate: no monotypic B-cell population. Iron stores adequate.
DIAGNOSIS: No involvement by lymphoma.
```

chart/11_2026-09-16_labs.txt

```
LABORATORY RESULTS: Repeat chemistry
Collected: 2026-09-16    Synthetic Clinical Laboratory

Test                                  Result  Flag  Units           Reference
Potassium                                5.0        mmol/L          3.5-5.1
Creatinine                               1.5  H     mg/dL           0.7-1.3
eGFR (CKD-EPI 2021)                       50  L     mL/min/1.73m2   >=60
Calcium                                  9.2        mg/dL           8.6-10.3
Phosphorus                               4.6  H     mg/dL           2.5-4.5
Uric acid                               10.4  H     mg/dL           3.4-7.0
LDH                                    1,290  H     U/L             135-225
```

chart/12_2026-09-17_progress_note.txt

```
HEMATOLOGY-ONCOLOGY FOLLOW-UP (RESULTS VISIT, PRE-TUMOR BOARD)
Date: 2026-09-17    Author: Synthetic Hematologist-Oncologist

Results reviewed with the patient, his wife and daughter. Biopsy shows DLBCL; FISH addendum received.
Abdominal pain is worse; he is now taking ibuprofen 400 mg four times daily. Night sweats persist.
Weight 70.2 kg. Still independent in self-care, resting much of the afternoon. No headache, vomiting,
confusion or new neurological symptoms. Urine output normal per patient.

Exam: T 37.6 C, HR 92, BP 122/74. Unchanged abdominal tenderness. Neuro exam non-focal.

Repeat labs 2026-09-16 reviewed (see results).

Patient goals: wants treatment aimed at cure. His granddaughter's wedding is in mid-December and he
very much wants to attend. Concerned about the 45-minute drive for frequent visits.

Case to be presented at lymphoma tumor board 2026-09-18 for a treatment plan; no treatment started yet.
```

C. Reference answer (hidden from the model)

diagnosis

Diffuse large B-cell lymphoma, NOS, germinal-center B-cell (GCB) phenotype by the Hans algorithm (CD10+, BCL6+, MUM1-). EBER negative; CD5 negative.

stage

Lugano (Ann Arbor) stage IV with B symptoms (drenching night sweats, ~10% weight loss): extranodal organ involvement of the right kidney, left adrenal gland and liver in addition to retroperitoneal/mesenteric nodal disease. Bone marrow negative, which does not change stage IV. Largest mass 9.4 cm, just below the 10 cm bulk definition used in Lugano.

biology

Not double-expressor: MYC 20% and BCL2 30% by IHC are below the 40%/50% thresholds. Not double-hit: FISH shows no MYC, BCL2 or BCL6 rearrangement, so this is not high-grade B-cell lymphoma with MYC and BCL2 rearrangements. Ki-67 ~70%.

prognosis

IPI 5 of 5, high risk: age >60, LDH elevated (1,290 U/L, >5x ULN), ECOG 2, stage IV, and more than one extranodal site (kidney, adrenal, liver). CNS-IPI 6 of 6 (adds kidney/adrenal involvement), high risk of CNS relapse (~10-12% at 2 years).

first_line_treatment

Curative-intent immunochemotherapy. Recommend Pola-R-CHP for 6 cycles as preferred (IPI >=2, age 18-80, ECOG 2 were POLARIX-eligible); R-CHOP for 6 cycles is an acceptable alternative. Start promptly given rising LDH and creatinine. Give a steroid prephase of prednisone 100 mg daily for 5-7 days before cycle 1 because of age >60, ECOG 2 and high tumor burden; it lowers first-cycle toxicity and often improves performance status. Full doses are appropriate: ECOG 2 is disease-related, so a dose-reduced regimen is not indicated. DA-EPOCH-R is not indicated.

cns_risk

High CNS-relapse risk because of kidney/adrenal involvement and CNS-IPI 6. Before or with cycle 1: contrast MRI brain and lumbar puncture with CSF cytology and flow cytometry to exclude occult CNS disease. CNS prophylaxis with high-dose methotrexate is an individualized option; recent data question its benefit, which should be discussed with the patient. If chosen, defer it until renal function recovers and NSAIDs are stopped (creatinine 1.5, eGFR 50 with renal involvement makes HD-MTX unsafe now), then give 2 cycles at >=3 g/m2 intercalated or after immunochemotherapy. Intrathecal therapy alone is not adequate prophylaxis for parenchymal relapse.

response_assessment

FDG PET-CT at the end of treatment (6-8 weeks after the last cycle), read with Lugano 2014 criteria and the Deauville 5-point scale; Deauville 1-3 is a complete metabolic response. An interim PET-CT after cycles 2-4 is optional and should not change therapy unless it shows progression. Biopsy residual Deauville 4-5 uptake if it would change management.

pretreatment_workup

Hepatitis B serology done: HBsAg and anti-HBc nonreactive (anti-HBs reactive from vaccination), so no antiviral prophylaxis is needed before rituximab. HCV and HIV nonreactive. Echo LVEF 60%, so anthracycline is permitted. Outstanding: contrast MRI brain, lumbar puncture with CSF cytology/flow, G6PD level before rasburicase, and urinalysis/urine protein to assess the creatinine rise.

supportive_care

High tumor lysis syndrome risk (uric acid 10.4, LDH >5x ULN, creatinine rising 1.1 -> 1.5, potassium 5.0, phosphate 4.6). TLS prophylaxis: admit for cycle 1 (or the prephase), with aggressive IV hydration, rasburicase rather than allopurinol given hyperuricemia with renal impairment (check G6PD first), and TLS labs (K, phosphate, calcium, uric acid, creatinine, LDH) every 6-12 hours for 72 hours. Stop ibuprofen and all NSAIDs now (nephrotoxic with rising creatinine; they interact with methotrexate); use acetaminophen plus an opioid for pain. Hold lisinopril during hydration/TLS risk. Primary G-CSF prophylaxis (pegfilgrastim) with every cycle, since age >=65 (an on-body injector avoids an extra 45-minute trip). Prednisone will cause hyperglycemia in this diabetic patient: monitor glucose, use an insulin correction plan, and hold metformin during the AKI and IV hydration. Antiemetics per regimen. PJP prophylaxis is not routinely needed; note the sulfonamide allergy if it is ever considered.

fitness_and_goals

ECOG 2 is disease-related (pain, B symptoms, tumor burden) and expected to improve with the prephase and treatment, so it is not a reason to de-escalate curative therapy; there are no significant cardiac or other comorbidities limiting full-dose therapy. The patient's goal is cure, and full-dose therapy is aligned with it. Six 21-day cycles starting late September would finish around the mid-December wedding; discuss timing so the nadir avoids the event. Arrange transport support for the 45-minute drive.

missing_information

• Contrast MRI brain and CSF cytology/flow cytometry (CNS staging not yet done)
• G6PD status before rasburicase
• Urinalysis and urine protein to assess the cause of the creatinine rise (renal involvement vs NSAID/ACE inhibitor effect)
• Patient's view on CNS prophylaxis after an informed discussion of uncertain benefit

D. Grading rubric (hidden from the model)

Rubric dlbcl_0003.v2.0. Must and should items total 100 points; pass threshold 0.75. A **dangerous** item fails the case (reward 0) when a must item is missed or a must-not item is committed. Points for must-not items are deductions.

ID	Kind	Severity	Pts	Criterion (what the judge reads)
dx_dlbcl_gcb	must	important	6	The response states the diagnosis is diffuse large B-cell lymphoma (NOS) with a germinal-center B-cell (GCB) phenotype.
stage_iv	must	dangerous	8	The response stages the disease as Lugano/Ann Arbor stage IV.
stage_extranodal_sites	should	minor	3	The response attributes stage IV to extranodal organ involvement of the kidney, adrenal gland and liver.
stage_b_symptoms	should	minor	2	The response notes B symptoms (night sweats and/or weight loss >10% or fevers).
mn_stage_below_iv	must_not	dangerous	0	The response assigns this patient a stage lower than IV (stage I, II or III).
mol_not_de_not_dh	must	important	6	The response states the lymphoma is neither double-expressor (MYC 20%, BCL2 30% below thresholds) nor double-hit (FISH negative for MYC, BCL2 and BCL6 rearrangements).
mn_calls_double_hit	must_not	dangerous	0	The response classifies this lymphoma as double-hit / high-grade B-cell lymphoma with MYC and BCL2 rearrangements.
mn_calls_double_expressor	must_not	important	5	The response classifies this lymphoma as double-expressor.
prog_ipi_high	must	important	6	The response calculates IPI as 5 (or explicitly high-risk IPI 4-5): age >60, elevated LDH, ECOG >=2, stage IV, >1 extranodal site.
prog_cns_ipi_high	must	important	6	The response calculates CNS-IPI as 6 or high risk (4-6), counting kidney/adrenal involvement.
tx_curative_pola_or_rchop	must	dangerous	8	The response recommends curative-intent full-dose immunochemotherapy with Pola-R-CHP or R-CHOP for 6 cycles.
tx_pola_preferred	should	minor	3	The response names Pola-R-CHP as the preferred regimen given IPI >=2 (R-CHOP acceptable alternative).
tx_prephase_steroid	must	important	4	The response recommends a steroid prephase (e.g. prednisone ~100 mg daily for 5-7 days) before cycle 1 given age >60, ECOG 2 and high tumor burden.
mn_non_curative	must_not	dangerous	0	The response recommends palliative-only treatment, best supportive care, or hospice as the plan.
mn_da_epoch	must_not	important	5	The response recommends DA-EPOCH-R or another intensified regimen for this patient.
mn_dose_reduced_initial	must_not	important	4	The response recommends a dose-reduced regimen (e.g. R-mini-CHOP) as the initial plan, despite age 67 and disease-related ECOG 2.
cns_risk_addressed	must	dangerous	4	The response identifies a high risk of CNS relapse driven by kidney/adrenal involvement and high CNS-IPI.
cns_staging_mri_csf	must	important	5	The response recommends baseline CNS evaluation: contrast MRI brain and lumbar puncture with CSF cytology and flow cytometry.
cns_prophylaxis_renal	should	important	4	The response discusses CNS prophylaxis (e.g. high-dose methotrexate) as an individualized option with uncertain benefit and ties its feasibility to renal function / stopping NSAIDs.
mn_hdmtx_now_despite_aki	must_not	important	4	The response schedules high-dose methotrexate immediately (e.g. with cycle 1) without addressing the rising creatinine, renal involvement, or concurrent NSAID use.
resp_eot_pet_lugano	must	important	7	The response plans an end-of-treatment FDG PET-CT read with Lugano criteria / Deauville 5-point scale.
resp_ds45_biopsy	should	minor	2	The response says residual Deauville 4-5 uptake at end of treatment should be biopsied if it would change management.

ID	Kind	Severity	Pts	Criterion (what the judge reads)
sup_tls_prophylaxis	must	dangerous	7	The response recognizes high tumor-lysis risk (uric acid 10.4, LDH >5x ULN, rising creatinine) and orders TLS prophylaxis with IV hydration, a uric-acid-lowering agent and frequent TLS labs.
sup_rasburicase_g6pd	should	important	3	The response favors rasburicase (given hyperuricemia with renal impairment) and checks G6PD before giving it.
sup_gcsf	must	important	4	The response recommends primary G-CSF prophylaxis (e.g. pegfilgrastim) with each cycle, given age >=65.
sup_stop_nsaid	should	important	3	The response stops ibuprofen/NSAIDs (nephrotoxic with rising creatinine; interacts with methotrexate) and offers alternative analgesia.
sup_steroid_glucose	should	minor	2	The response anticipates steroid-induced hyperglycemia in this diabetic patient (glucose monitoring / insulin plan) or adjusts metformin for falling eGFR.
work_hbv_reviewed	should	minor	2	The response confirms hepatitis B serology was checked (HBsAg and anti-HBc nonreactive) so no antiviral prophylaxis is needed before rituximab.
work_echo_reviewed	should	minor	1	The response notes the baseline LVEF of 60% permits anthracycline.
fit_ecog_disease_related	must	important	4	The response judges ECOG 2 as disease-related and not a reason to de-escalate curative therapy, and acknowledges the patient's goal of cure.

E. Grader self-tests (regex mode, recomputed now)

Each adversarial answer must produce its expected verdict. 01_reference is the gold answer.

Answer	Reward	Passed	Expected	Dangerous errors fired
01_reference	1.00	yes	pass	-
bad_double_hit_da_epoch	0.00	no	fail	mn_calls_double_hit
bad_no_tls	0.00	no	fail	sup_tls_prophylaxis
bad_palliative	0.00	no	fail	tx_curative_pola_or_rchop, mn_non_curative
bad_r_mini_chop	0.00	no	fail	tx_curative_pola_or_rchop
bad_stage_iii	0.00	no	fail	mn_stage_below_iv
v1_gold_answer	0.00	no	fail	sup_tls_prophylaxis

F. Reviewer sign-off

Reviewer	Credentials	Date	Verdict	Changes requested